Panitumumab + FOLFIRI

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Panitumumab + FOLFIRI

Active medicines: panitumumab + irinotecan + folinic acid + fluorouracil

Emetogenic risk: moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally	Day 1 before chemotherapy Dose may be adjusted for individual/QT risk.
Dexamethasone	8 mg orally daily	Days 1–3 Reconcile all steroid prescriptions.
Metoclopramide	10 mg orally up to three times daily as prescribed	As required on subsequent days Confirm suitability, duration and contraindications.
Atropine	250 micrograms subcutaneously	Before irinotecan per National Cancer Control Programme (NCCP) prophylaxis Check contraindications including glaucoma; this does not treat delayed diarrhoea. Use caution in asthma, as specified in the NCCP acute cholinergic-syndrome advice.

Additional phase-specific support

- No routine panitumumab premedication unless previous hypersensitivity/local policy indicates.
- Written delayed-diarrhoea plan and urgent fever/dehydration advice; do not confuse with acute cholinergic management.
- Skin care and electrolytes; monthly Mg/Ca/electrolytes for two months after last panitumumab.
- Supply a written irinotecan delayed-diarrhoea rescue prescription: NCCP reference loperamide 4 mg initially, then 2 mg every 2 hours; continue for 12 hours after the last liquid stool, never longer than 48 hours at this high-dose schedule. Start advised electrolyte-containing fluids at the first liquid stool and contact the team early. Fever, dehydration, severe pain or persistent symptoms require urgent oncology review.
- Irinotecan treatment-related diarrhoea must fully resolve before restart; a table-grade threshold alone does not clear the next dose.
- If treatment is delayed for cytopenia, follow the source-specific weekly blood-count checks and maximum delay; assess the antibody and chemotherapy components separately when indicated.
- Medication review: NCCP contraindicates concomitant brivudine, sorivudine and analogues with fluorouracil; review dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD)-affecting medicines, phenytoin toxicity/levels and warfarin international normalised ratio (blood-clotting test) (INR).
- Review CYP3A inducers/inhibitors and grapefruit; avoid prochlorperazine on the same day as irinotecan. Gilbert syndrome warrants consideration of a reduced starting irinotecan dose; no universal percentage is supplied.
- EMA Vectibix also requires interruption for intolerable dermatologic reactions; a grade below 3 does not automatically clear treatment.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00448 v5 · p6 · NCCP NCIS Medical Oncology antiemetics V8, Table 1

Sources checked 2026-11-05 · SACTCheck v0.84.35 · Regimen-level drafts; prescriber and oncology-pharmacy verification required